

SKIN & LASER

# Laser Hair Removal Protocol

Wavelength and parameter selection by skin type, test spotting, endpoints and interval planning

|                                   |                           |                                |                                  |
|-----------------------------------|---------------------------|--------------------------------|----------------------------------|
| DOCUMENT<br>SKN-001               | VERSION<br>1.0            | EFFECTIVE<br>January 2026      | REVIEW CYCLE<br>Annual           |
| APPLIES TO<br>All laser operators | OWNER<br>Medical Director | CLASSIFICATION<br>Confidential | SUPERSEDES<br>All prior versions |

## 1 Purpose & Scope

This protocol governs laser and light-based hair reduction across all device platforms. It covers device and parameter selection against skin type, the screening that prevents most burns, mandatory test spotting, treatment endpoints, and interval planning by anatomical area.

THREE FAILURES CAUSE ALMOST EVERY BURN

Parameters wrong for the skin type. Test spot skipped. A photosensitizing medication or recent tanning nobody asked about. All three are prevented by the checklist in this protocol, and all three appear repeatedly in complaint files.

MANUFACTURER IFU GOVERNS

This protocol provides the selection framework and the safety process. The specific fluence, pulse duration, spot size and cooling settings for your device come from the manufacturer's instructions for use and from your medical director. Never copy numeric settings from a template, a colleague at another clinic, or a training course on a different platform.

## 2 Wavelength Selection

Longer wavelengths penetrate deeper and are absorbed less by epidermal melanin, which makes them safer in darker skin. Shorter wavelengths are more strongly absorbed by melanin, which makes them more effective on fine or light hair in fair skin and more dangerous in pigmented skin.

GENERAL SUITABILITY — CONFIRM AGAINST YOUR DEVICE IFU

| Wavelength      | Typical platform     | Best suited to                                               | Caution                                                                          |
|-----------------|----------------------|--------------------------------------------------------------|----------------------------------------------------------------------------------|
| 755 nm          | Alexandrite          | Fitzpatrick I–III with fine to medium hair. High efficacy.   | High melanin absorption. Generally avoided in Fitzpatrick IV and above.          |
| 810 nm          | Diode                | Fitzpatrick I–IV. The most versatile general-purpose option. | Use longer pulse durations and robust cooling as skin type darkens.              |
| 1064 nm         | Nd:YAG               | Fitzpatrick IV–VI. The safest option in darker skin.         | Less efficient on fine or light hair. More painful; plan anesthesia and cooling. |
| Broadband (IPL) | Intense pulsed light | Fitzpatrick I–III only.                                      | See SKN-002. Not used for hair reduction in Fitzpatrick IV–VI in this practice.  |

**PARAMETER DIRECTION OF TRAVEL**

As skin type darkens: longer wavelength, longer pulse duration, lower fluence, more aggressive epidermal cooling, and a more conservative starting point. As hair becomes finer or lighter: efficacy falls across all platforms, and this must be discussed before the patient purchases a package. White, grey and true blonde hair does not respond.

### 3 Screening & Exclusions

- ☐ Fitzpatrick assessment completed and documented per SKN-007.
- ☐ Photosensitizing medication screen — including but not limited to isotretinoin, tetracyclines, St John's Wort, amiodarone, thiazides, sulfonamides and topical retinoids.
- ☐ Recent sun exposure, tanning bed use or self-tanner within the exclusion windows set by the medical director.
- ☐ Active infection, open lesion, eczema or psoriasis in the treatment area.
- ☐ History of keloid or hypertrophic scarring.
- ☐ History of herpes simplex where treating perioral or bikini areas.
- ☐ Pregnancy — practice policy set by the medical director; most defer treatment.
- ☐ Recent chemical peel, resurfacing or injectable treatment in the same area.
- ☐ Tattoos or permanent makeup in the field — these are avoided, not treated over.
- ☐ Photosensitizing conditions, connective tissue disease, or a history of photo-exacerbated disorder.
- ☐ Gold therapy — an absolute contraindication for most laser treatment.
- ☐ Realistic expectation discussion completed: reduction rather than permanent removal, multiple sessions required, maintenance likely.

**ISOTRETINOIN**

The traditional six-month washout is increasingly debated in the literature, but it remains the conservative standard and the position most easily defended. Your medical director must set the practice interval explicitly and record the reasoning. Do not let individual operators decide this case by case.

### 4 Test Spotting

**MANDATORY — NO EXCEPTIONS**

A test spot is required for every new patient, and again whenever the device, the wavelength, or the treatment area changes materially. A returning patient being treated on a newly acquired platform is a new patient for this purpose. 'They've had it before' is not a substitute.

- 1 Select a discreet site** within, or immediately adjacent to, the intended treatment area so the skin is representative.
- 2 Apply the intended treatment parameters** — a test at settings you do not plan to use tells you nothing.
- 3 Deliver a small number of pulses** and photograph the site.

- 4 **Observe the immediate response** for several minutes. Perifollicular erythema and mild edema is the desired endpoint.
- 5 **Wait the interval** set by your medical director before full treatment. Delayed reactions are the ones that matter, and a longer interval is used for Fitzpatrick IV–VI.
- 6 **Review the site** and record the finding. Go, no-go, or adjust parameters.
- 7 **Document** site, parameters, immediate response, review date and outcome.

| Test spot response                               | Interpretation                                                                    |
|--------------------------------------------------|-----------------------------------------------------------------------------------|
| Perifollicular erythema and mild edema           | Desired endpoint. Proceed.                                                        |
| No visible response                              | Under-treatment. Reassess parameters with the medical director before increasing. |
| Immediate greying or whitening of the epidermis  | Epidermal injury. Stop. Do not treat at these settings.                           |
| Blistering, crusting or pigment change at review | Excessive thermal injury. Do not proceed. Medical director review required.       |
| Prolonged or severe pain during the pulse        | Excessive energy. Reduce and reassess.                                            |

## 5 Treatment Procedure

- 1 **Confirm consent, screening and test spot outcome** before the device is powered on.
- 2 **Eyewear on everyone in the room** — patient, operator and observers — at the correct optical density for the wavelength in use. See SKN-008.
- 3 **Prepare the area.** Shaved, clean, dry, free of makeup, deodorant, lotion and residue. Do not treat unshaved skin; surface hair causes epidermal burns and singeing.
- 4 **Confirm parameters against the chart and the IFU**, and confirm the skin type recorded matches the patient in front of you.
- 5 **Engage cooling** — contact, cryogen or air — per the device requirements.
- 6 **Treat methodically** in a defined pattern with appropriate overlap. Track coverage so no area is double-pulsed inadvertently.
- 7 **Monitor the endpoint continuously.** Stop immediately on any greying, whitening, blistering or disproportionate pain.
- 8 **Post-treatment cooling** and a bland emollient as directed.
- 9 **Written aftercare** supplied and explained: sun avoidance, no heat or friction, no active skincare, and warning signs with contact instructions.

**10** **Record** parameters, passes, total pulses, areas treated, endpoint observed and any adverse response.

**6 Intervals, Series & Paradoxical Hypertrichosis**

| Area                       | Typical interval       | Note                                                          |
|----------------------------|------------------------|---------------------------------------------------------------|
| Face and neck              | Shorter intervals      | Faster hair cycle. Higher paradoxical hypertrichosis risk.    |
| Axilla and bikini          | Intermediate intervals | Responds well; commonly the best outcomes.                    |
| Legs, arms, back and chest | Longer intervals       | Slower cycle; lengthen the interval as the series progresses. |

Set the specific intervals with your medical director. Extend the interval as response improves rather than treating on a fixed calendar — treating too soon wastes a session because hair is not in the growth phase.

**PARADOXICAL HYPERTRICHOSIS**

A minority of patients develop increased fine hair growth in or adjacent to the treated area, most often on the face and neck and more commonly in Fitzpatrick III–VI. Disclose this risk in consent before the first treatment, not after it appears. If it occurs, photograph it, notify the medical director, and reassess the treatment plan rather than simply increasing energy.

**7 Quick Reference**

QUICK REFERENCE CARD

**Before every LHR pulse**

- Fitzpatrick documented.** Parameters match the skin type in front of you.
- Meds and tanning screened** today, not at the first visit.
- Test spot done and reviewed** — new patient, new device, or new area.
- Eyewear on everyone.** Correct OD for this wavelength.
- Skin shaved, clean, dry.** Never treat unshaved.
- Cooling engaged.** Confirm before the first pulse.
- Endpoint = perifollicular erythema.** Greying or whitening → stop.
- Record parameters, passes and endpoint** every session.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

|                                               |                                 |
|-----------------------------------------------|---------------------------------|
| PRACTICE NAME                                 | LOCATION / SITE                 |
| MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS | LICENSE NUMBER & STATE          |
| SIGNATURE                                     | DATE ADOPTED / NEXT REVIEW DATE |

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